

Avoidant/Restrictive Food Intake Disorder

What is Avoidant/Restrictive Food Intake Disorder (ARFID)?

Avoidant/Restrictive Food Intake Disorder (ARFID) is a serious eating disorder where a person restricts their dietary intake and struggles to obtain adequate nutrition. Unlike other eating disorders, such as anorexia nervosa and bulimia nervosa, people with ARFID **do not** restrict their eating because of fears of weight gain or body image concerns.

People with ARFID may have significant nutritional deficiencies, may be weight-suppressed, or may be dependent on tube feeding or oral supplement drinks. ARFID can have negative effects on people's functioning and psychological wellbeing.

While ARFID has a fairly low prevalence rate, it can affect anyone regardless of their gender or age.

People with ARFID can be misunderstood as 'stubborn', 'fussy', or 'picky eaters'. However, it's much more complex than this. Research suggests there are 3 types of ARFID presentations; people may fall into one, two, or all three of these categories.

- **Lack of interest in eating or food:** Some people with ARFID undereat as they don't feel hungry or aren't interested in food. They might experience poor appetite or have little awareness of their hunger and fullness cues. They might experience becoming full very quickly or feel physically uncomfortable after eating. Some may describe eating as 'a chore' and get little enjoyment from food.
- **Fear of aversive consequences:** Some people with ARFID avoid eating because they are afraid something bad will happen if they do. This could include a fear of choking on food, having an allergic reaction, or experiencing nausea or vomiting. This can lead to limiting what or how much they eat in an attempt to avoid these consequences.
- **Sensory sensitivities:** Some people with ARFID avoid certain foods due to sensory characteristics such as texture, smell, taste, or appearance. These individuals may experience a disgust response when trying new foods and find this experience to be overwhelming or unpleasant. They often feel more comfortable eating a small range of foods that they are familiar with instead.

What causes ARFID?

Like all eating disorders, there is no single cause of ARFID. It typically develops as a result of a complex interplay between genetics and environmental factors. For many, ARFID develops in infancy and early childhood. While 'picky eating' is more common in young children (often peaking around 2 – 6 years old), this typically decreases as people age. However, a small number of these children who started out as 'picky eaters' may go on to develop ARFID later in life. For others, ARFID may develop after a scary or distressing experience with eating at any point in their life.



Impacts of ARFID

ARFID can have serious effects on someone's physical health and mental wellbeing.

- **Physical health:** For some, ARFID can lead to becoming weight suppressed. For children and adolescents, this may be seen as a failure to meet expected weight gain/growth for their stage of development. Some individuals may develop significant nutritional deficiencies due to the limited variety of foods they consume. In addition, individuals may display symptoms of [Starvation Syndrome](#), such as reduced muscle mass, loss of periods (in those who menstruate), and increased risk of osteoporosis.
- **Psychological wellbeing:** People with ARFID may feel embarrassed or upset by their eating, leading to self-critical thoughts, and feelings of guilt, shame, and anxiety. They might be afraid to eat at different restaurants due to worries about what food would be available. Some may avoid eating in front of other people due to worries of negative judgement from others. This can have a negative impact on people's mood and quality of life in a range of areas (such as their relationships, work, hobbies, etc.).

Just as with any anxiety disorder, the more we avoid something we feel anxious about, the more that fear persists and grows stronger. Starvation syndrome can also impact the brain in ways that make it difficult to try new or anxiety-provoking things. This can keep people feeling stuck.

Due to these significant physical and psychological effects, it is important for people with ARFID to access treatment.

Treatment for ARFID

Cognitive Behavioural Therapy (CBT) has been adapted as a treatment for ARFID in both children and adults. This is known as CBT-AR and has shown to be an effective treatment in helping people recover.



Treatment involves understanding and developing regular eating patterns, improving nutritional intake, and restoring weight (if necessary) to a range that is healthy for one's body. It also helps people test and overcome their fears through repeated exposure to trying new foods, and developing strategies to help manage distress around eating. While it can be challenging, full recovery is possible and worthwhile.

If you would like help for ARFID, we recommend speaking to your doctor and seeking a referral to see a Clinical Psychologist or Dietitian who can deliver CBT-AR.

Please note that this handout is not a substitute for treatment for ARFID. ARFID is a serious eating disorder and getting the right support is important.

If you or someone close to you has signs of an eating disorder it is important to see a GP and discuss seeking help as soon as possible.